



Harbourview Residential Care

Complaints, Feedback and Open Disclosure Policy

Raising, resolving and learning from concerns about care

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This is a fictional document created to demonstrate document-summarisation software. The provider, facilities, personnel, incident references and document identifiers named in it do not exist. It is not a record of any real organisation and must not be relied on as compliance advice.

Document control

This document is controlled by the Head of Consumer Experience. Printed copies are uncontrolled and must be checked against the document register before use. Proposed amendments are submitted to the document owner and considered by the Clinical Governance Committee at its next scheduled meeting.

Version history

Version	Date	Author	Summary of change
3.0	12 Sep 2024	R. Delaney	Introduced the three-tier resolution pathway.
4.0	4 Apr 2025	R. Delaney	Added anonymous and third-party complaint handling.
4.2	17 Jan 2026	S. Bhattacharya	Added translated complaint materials in six languages.
5.0	28 Aug 2026	S. Bhattacharya	Rewritten against the Strengthened Standards; added the open disclosure interface at Part 5.

Distribution and accessibility

The current version is published on the provider intranet and is available in hard copy at each nurses' station. Alternative formats, including large print and interpreted summaries, are provided on request to the document owner.

Audience	Method	Frequency
All care and clinical workers	Intranet and induction pack	On publication
Registered and enrolled nurses	Clinical handover briefing	On publication
Consumer Advisory Body	Emailed summary	Quarterly
Contracted and agency staff	Agency portal	Before first shift

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Part 1 Purpose and commitment

Harbourview Residential Care treats a complaint as information the provider needs and would not otherwise have. A complaint is not a nuisance and is not a reflection on the worker who receives it. This document sets out how anyone may raise a concern, how it will be handled, and what the provider will do with what it learns.

The provider commits that no person will experience any adverse consequence for making a complaint. Retaliation against a person who complains, or against a worker who supports them, is a serious disciplinary matter and, where it affects a person receiving care, a reportable incident.

1.1 Who may complain

- A person receiving care, at any time and about anything.
- A supporter, family member, friend or advocate, with or without the person's involvement.
- A worker, contractor, volunteer or student.
- A visitor, a health practitioner, or any member of the public.
- Any person anonymously, including through the anonymous feedback line.

1.2 What may be complained about

Anything. There is no category of concern that falls outside this document, including the conduct of senior staff, the conduct of the Board, the quality of food, the condition of the building, billing, and the handling of an earlier complaint.

1.3 Barriers the provider must remove

The provider recognises that people receiving aged care often do not complain because they fear consequences, do not want to be a burden, or do not know how. Workers must therefore actively invite feedback rather than wait for it, and must treat an expression of dissatisfaction as a complaint even when the person says it is 'not a complaint'.

Part 2 How a complaint may be made

A complaint may be made in any way the person finds easiest. The provider must not require a particular form, a written statement, or a formal word such as 'complaint' before acting.

2.1 Channels

Channel	Detail	Monitored
In person	To any worker, at any time	Continuously
Feedback form	Available at reception, in each wing and on the website	Daily
Telephone	The published feedback line	Business hours, with voicemail
Email	The published feedback address	Daily
Anonymous	Locked feedback boxes in each wing and the anonymous online form	Weekly
Resident and relative meetings	Standing agenda item at every meeting	Monthly

2.2 Support to make a complaint

The provider offers an interpreter, an advocate, or assistance to write down a concern, at no cost to the person. Contact details for the Older Persons Advocacy Network and the Aged Care Quality and Safety Commission are displayed in every wing and given to every person on entry to care.

2.3 The right to complain externally at any time

A person may complain directly to the Aged Care Quality and Safety Commission at any time, whether or not they have raised the matter with the provider first, and whether or not an internal complaint is still open. Workers must never discourage or delay an external complaint, and must offer to help the person make one if they wish.

2.4 Anonymous complaints

An anonymous complaint is investigated on its merits so far as the information allows. The absence of a named complainant does not reduce the provider's obligation to act, and the outcome is recorded and reported to the committee in the same way as any other complaint.

Part 3 Acknowledgement, triage and resolution

Every complaint is acknowledged, assessed for risk, and resolved as close to the point of care and as quickly as the matter allows.

3.1 Timeframes

Step	Timeframe	Responsible
Acknowledge receipt to the complainant	Within 1 business day	Worker receiving the complaint
Assess risk and assign a resolution tier	Within 1 business day	Head of Consumer Experience
Resolve a Tier 1 concern at the point of care	Within 5 business days	Facility Manager
Resolve a Tier 2 complaint	Within 20 business days	Head of Consumer Experience
Resolve a Tier 3 complaint requiring investigation	Within 30 business days	Chief Executive Officer
Provide a written outcome to the complainant	Within 5 business days of resolution	Head of Consumer Experience
Advise the complainant of any delay and the reason	Before the due date passes	Head of Consumer Experience

3.2 Risk assessment on receipt

Every complaint is assessed on receipt for indicators of abuse, neglect, unexplained injury, financial exploitation or immediate risk to any person. Where any such indicator is present, the matter is escalated immediately under the Incident Management Policy in parallel with the complaint process; the two processes run together and neither waits for the other.

3.3 Resolution tiers

Tier 1 covers concerns that can be resolved immediately by the worker or manager receiving them, such as a meal preference or a room temperature. Tier 2 covers matters requiring inquiry across a shift or a team. Tier 3 covers matters involving alleged misconduct, serious harm, or a systemic failure, and is always investigated by a person independent of the area concerned.

3.4 Keeping the complainant informed

The complainant is given a named contact and told what will happen next and when. Where a matter cannot be resolved within the timeframe, the complainant is told before the due date passes, given a reason and a revised date. Silence is never an acceptable response to a delay.

Part 4 Outcomes, remedies and review

A complaint is resolved when the provider has determined what happened, told the complainant, taken any action required, and offered a remedy where one is appropriate.

4.1 Remedies

- An apology and an explanation of what happened and why.
- A change to the person's care plan, room, worker allocation or routine.
- A refund, fee waiver or correction to an account.
- Repair, replacement or reimbursement for lost or damaged property.
- A change to a procedure, roster or training program where a system issue is found.

4.2 Written outcome

The written outcome states what was complained about, what the provider found, what it has done or will do, by when, and how the complainant may seek review if dissatisfied. It is written in plain language and translated where the person's preferred language is not English.

4.3 Internal review

A complainant who is dissatisfied with an outcome may request an internal review within 30 days. The review is conducted by a senior person who had no involvement in the original handling, and is completed within 20 business days. The reviewer may confirm, vary or overturn the original outcome.

4.4 Unreasonable conduct

In rare cases a complainant's conduct may be unreasonable, for example repeated contact after a matter is exhausted, or abusive behaviour toward workers. Restrictions on contact may be applied only with the written approval of the Chief Executive Officer, must be proportionate and time-limited, must be notified in writing with reasons, and must never restrict the person's access to care or their right to complain externally.

Part 5 Open disclosure and learning from complaints

Where a complaint reveals that a person has been harmed, the provider applies open disclosure in addition to resolving the complaint. Resolving a complaint administratively does not discharge the obligation to speak openly with the person harmed.

5.1 Trend analysis

Complaints are coded by category, service area and theme, and analysed monthly. The analysis looks for patterns that individual complaints do not reveal: repeated concerns about one shift, one worker, one wing, or one time of day. Trends are reported to the committee with the provider's proposed response.

5.2 Closing the loop

Improvements arising from complaints are published quarterly in the 'You said, we did' bulletin, which is displayed in every wing, emailed to supporters, and tabled at resident and relative meetings. The next bulletin is due for publication on 15 January 2027.

5.3 Consumer Advisory Body

The Consumer Advisory Body reviews de-identified complaints data at each meeting and gives the Board its own view on whether the provider is listening. The Board must respond in writing to any concern the Advisory Body raises, within 30 days.

Part 5A Feedback that is not a complaint

Compliments, suggestions and observations are recorded through the same system as complaints, so that the provider hears what is working as well as what is not. Compliments are passed to the worker or team named and to their manager within five business days.

5A.1 Actively seeking feedback

The provider seeks feedback rather than waiting for it, through the annual consumer experience survey, quarterly short-form pulse surveys, monthly resident and relative meetings, and structured conversations at each care plan review. Response rates and results are reported to the Board, and the annual survey for the current cycle closes on 30 November 2026.

Part 6 Governance and accountability for complaints and feedback

The Board of Harbourview Residential Care holds ultimate accountability for complaints and feedback and discharges that accountability through the Board Quality and Safety Sub-Committee. The Board receives a standing report at each ordinary meeting covering performance against the indicators set out in Part 9, together with any matter the Head of Consumer Experience considers requires the Board's attention before the next reporting cycle.

The Board Quality and Safety Sub-Committee meets monthly. Its quorum is four members and must include at least one registered nurse and one member independent of the operational management line. Minutes are retained for seven years and are made available to the Aged Care Quality and Safety Commission on request.

6.1 Allocation of responsibilities

Responsibility for the requirements of this document is allocated as set out below. Allocation of a responsibility does not relieve any worker of their own professional and legal obligations.

Role	Responsibility
Board	Receives complaints trends quarterly; responds in writing to Consumer Advisory Body concerns.
Chief Executive Officer	Approves any restriction on complainant contact; oversees Tier 3 matters.
Head of Consumer Experience	Owens this document; triages complaints; maintains the register; reports trends.
Facility Manager	Resolves Tier 1 concerns; ensures feedback is actively invited on every shift.
All workers	Receive complaints willingly; record them the same day; never discourage an external complaint.
Consumer Advisory Body	Reviews de-identified complaints data; advises the Board independently.

6.2 Delegation

The Head of Consumer Experience may delegate an operational function under this document in writing, recording the scope and duration of the delegation in the delegations register. A delegation must not exceed twelve months without review, and the delegate must hold the qualifications and current registration the function requires. The Head of Consumer Experience remains accountable for a delegated function.

6.3 Conflicts of interest

A worker who has a personal, financial or familial interest that could reasonably be perceived to influence a decision made under this document must declare that interest to the Head of Consumer Experience before taking part in the decision. Declared interests are recorded in the conflicts register and reviewed by the

Board Quality and Safety Sub-Committee each quarter.

Part 7 Workforce competence and training

No worker may perform a function governed by this document until they have completed the training set out in this Part and been assessed as competent. The Head of Consumer Experience maintains a training matrix mapping every role to its mandatory modules, and the workforce management system prevents rostering a worker to a function for which their competency has lapsed.

Module	Audience	Frequency	Assessment
Receiving a complaint well	All workers	Induction, then annually	Role-play observation
Complaint triage and risk assessment	Managers	Annually	Scenario assessment
Investigating a Tier 3 complaint	Senior managers	Every two years	Case-based assessment
Working with interpreters and advocates	All workers	Every two years	Online assessment
Recognising abuse disclosed in a complaint	All workers	Annually	Scenario assessment

7.1 Induction

New workers complete the complaints and feedback component of induction before their first unsupervised shift. Induction covers this document, the escalation pathway in Part 5, and the worker's obligation to report. A worker who has not completed induction may work only under the direct supervision of a competent worker.

7.2 Competency assessment and remediation

Competency is assessed by direct observation against the criteria at Appendix B, not by attendance alone. A worker assessed as not yet competent is placed on a supervised practice plan for a defined period, reassessed at the end of that period, and stood down from the relevant function if the second assessment is not satisfactory.

The Head of Consumer Experience reports training completion rates to the Board Quality and Safety Sub-Committee monthly. Where completion for any mandatory module falls below 95 per cent, the Head of Consumer Experience must provide the Board Quality and Safety Sub-Committee with a written recovery plan at the next meeting.

Part 8 Records, privacy and information management

Records created under this document form part of the care record and are managed in accordance with the provider's Records Management Policy and the Privacy Act 1988 (Cth). Entries must be contemporaneous, factual, legible and attributed to an identified author, and must not be altered after the fact except by a dated and attributed addendum.

8.1 Retention

Record	Minimum retention	Custodian
Complaint records and outcomes	7 years from closure	Head of Consumer Experience
Internal review decisions	7 years	Chief Executive Officer
Anonymous feedback	3 years	Head of Consumer Experience
Consumer experience survey data	5 years, de-identified	Quality Manager

8.2 Access and disclosure

An individual, or their supporter where authorised, may access records held about them on written request. Requests are actioned within 30 days. Personal information is disclosed outside the provider only with consent, or where disclosure is required or authorised by law, and every such disclosure is recorded in the disclosure register.

8.3 Information security

Access to the clinical information system is role-based and individually credentialled. Shared logins are prohibited. Access is revoked on the day a worker's engagement ends, and access logs are reviewed quarterly for anomalous patterns. A suspected data breach is escalated immediately under the Notifiable Data Breaches scheme.

Part 9 Monitoring, measurement and continuous improvement

Performance under this document is measured against the indicators below. The Head of Consumer Experience reports results to the Board Quality and Safety Sub-Committee monthly and to the Board quarterly. An indicator that falls outside its target for two consecutive months triggers a documented improvement action with a named owner and a due date.

Indicator	Target	Source	Frequency
Complaints acknowledged within 1 business day	100%	Complaints register	Monthly
Complaints resolved within their tier timeframe	at least 90%	Complaints register	Monthly
Complainants advised before a due date passes	100%	Register audit	Monthly
Complaints reopened after an outcome	below 5%	Complaints register	Quarterly
Consumer experience survey response rate	at least 60%	Survey platform	Annually
Improvements published in the quarterly bulletin	at least 4 per quarter	Bulletin archive	Quarterly

9.1 Internal audit

The Head of Consumer Experience conducts a documented internal audit of compliance with this document at least twice each year, using the tool at Appendix C. Findings are rated as conformity, opportunity for improvement, minor non-conformity or major non-conformity. A major non-conformity must be reported to the Board Quality and Safety Sub-Committee within five business days of identification.

9.2 Continuous improvement register

Improvement actions arising from audits, incidents, complaints, feedback and consumer advisory input are recorded in the continuous improvement register with an owner, a due date and an evidence field. The register is reviewed at each committee meeting and overdue items are escalated to the Board.

9.3 Consumer and workforce input

The provider seeks the views of individuals receiving care, their supporters and the workforce on the operation of this document at least annually, through the consumer experience survey, resident and relative meetings and workforce feedback channels. Results and the provider's response are published in the quarterly quality report.

Part 10 Risk management

Risks to the delivery of the requirements in this document are assessed using the enterprise risk matrix, which rates a risk on the likelihood of occurrence against the consequence for the individual receiving care. Risks rated high or extreme are recorded on the strategic risk register and reviewed by the Board each quarter.

Risk	Rating	Existing controls	Treatment
People do not complain for fear of consequences	High	Anonymous channels; anti-retaliation commitment	Confidential survey question added from 30 November 2026
A complaint disclosing abuse is handled only as a complaint	High	Risk screen on receipt; parallel incident process	Monthly audit of complaints for missed incident triggers
Complaints from people with communication difficulty go unheard	High	Interpreters, advocates, assisted recording	Translated materials in six languages by 28 February 2027
Timeframes missed without the complainant being told	Medium	Automated reminder before due date	Escalation report to CEO weekly from 1 December 2026
Trends not acted on across services	Medium	Monthly coded trend analysis	Cross-service theme review each quarter

10.1 Dignity of risk

The provider supports each individual to make their own decisions, including decisions that carry risk. Where an individual chooses a course of action a clinician considers unsafe, the discussion, the information given, the individual's decision and any agreed mitigations are documented in the care plan and reviewed at each care plan review. Supporting a person's choice is not a failure of care.

Part 11 Review of this document

This document is reviewed at least every two years, and sooner where a legislative change, a major non-conformity, a serious incident, a coronial finding or a substantive change to practice makes review necessary. The next scheduled review falls due on 1 October 2028.

The review is coordinated by the Head of Consumer Experience and must involve at least one registered nurse, one worker who applies the document in daily practice, and one consumer representative drawn from the Consumer Advisory Body. The reviewed document is approved by the Board Quality and Safety Sub-Committee and, where the change is material, by the Board.

11.1 Related documents

- HV-CORP-001 Incident Management Policy
- HV-CORP-008 Workforce Code of Conduct
- HV-CLIN-005 Open Disclosure Procedure
- HV-CORP-012 Privacy and Information Handling Policy
- HV-CORP-014 Consumer Advisory Body Terms of Reference

11.2 Legislative and standards context

- Aged Care Act 2024 (Cth) and the Aged Care Rules made under it
- Strengthened Aged Care Quality Standards, effective 1 November 2025
- Privacy Act 1988 (Cth) and the Australian Privacy Principles
- Work Health and Safety Act 2011 (Cth) and corresponding state legislation
- Aged Care Quality and Safety Commission Rules relating to complaints

Part 12 Communication, engagement and supported decision-making

Every person receiving care from Harbourview Residential Care has the right to be told, in a way they understand, what is happening in their care and why. Information provided under this document must be offered in the person's preferred language and format, and must be repeated or restated where comprehension is uncertain. Reliance on a nod or a signature is not evidence of understanding.

12.1 Interpreters and communication support

An accredited interpreter must be engaged where a person's preferred language is not English and the discussion concerns consent, risk, a change to care, an incident affecting them or a complaint. Family members must not be used as interpreters for these discussions except in an emergency, and where that occurs the reason is recorded and the discussion repeated with an accredited interpreter within two business days.

12.2 Supported decision-making

The provider presumes that every person has decision-making capacity. Where capacity is in question, the assessment must be decision-specific, time-specific and documented, and must not be inferred from a diagnosis, age or communication difficulty. A person's supporter assists the person to make and communicate their own decision; a substitute decision-maker acts only where the person cannot make the decision even with support.

12.3 Engagement with supporters and representatives

The provider maintains a current record of each person's nominated supporters and representatives, including the scope of any authority held and the instrument granting it. The record is confirmed with the person at each care plan review and whenever the person's circumstances change.

12.4 Culturally safe practice

Care is delivered in a way that is safe for the person's culture, spirituality, gender, sexuality and life experience. For Aboriginal and Torres Strait Islander people, the provider offers connection to community-controlled health services and to family and community, and records the person's wishes about who is involved in decisions about their care.

Part 13 Emergency, contingency and business continuity

The requirements of this document continue to apply during an emergency. Where an emergency makes ordinary compliance impossible, the provider must record what was done instead, why, and for how long, and must return to ordinary practice as soon as the emergency permits.

13.1 Planned system outages

Where the clinical information system is unavailable, workers record required information on the approved paper forms and enter it into the system within 24 hours of the system being restored. The paper record is retained and scanned into the care record; it is not destroyed after entry.

13.2 Surge, outbreak and staffing shortfall

During an infectious disease outbreak or a staffing shortfall, the Head of Consumer Experience may authorise temporary variations to non-statutory requirements of this document. Statutory obligations, including notification timeframes, cannot be varied. Every temporary variation is recorded in the variations log with the authorising officer, the reason, the start date and the review date, and is reported to the next committee meeting.

13.3 Evacuation and relocation

Where a person is relocated in an emergency, the receiving service must be given the person's current care plan, medication chart, advance care directive and supporter contact details before or with the person. A relocation is not complete until a handover has been given to and acknowledged by a named clinician at the receiving service.

Part 14 Subcontracted and external service providers

The provider remains fully accountable for care delivered on its behalf by a subcontractor, agency or external practitioner. Engaging another organisation transfers work; it does not transfer accountability.

14.1 Contractual requirements

- Every service agreement must require compliance with this document by name.
- Every agreement must require the subcontractor to report incidents to the provider immediately, and in any event within four hours of becoming aware.

- Every agreement must permit the provider to audit the subcontractor's compliance on reasonable notice.
- Every agreement must require workers supplied to hold current screening and the qualifications the role requires.
- Every agreement must be reviewed before renewal, and no agreement may be renewed where a material non-conformity remains unresolved.

14.2 Monitoring subcontractor performance

The provider reviews the performance of each subcontractor at least annually against the indicators at Part 9, and more frequently where a concern has been identified. The review considers incidents, complaints, workforce screening currency and audit findings. The next scheduled subcontractor review cycle concludes on 31 March 2027.

14.3 Agency and locum workers

An agency worker may not commence a shift until the provider has sighted evidence of current registration where the role requires it, a current worker screening clearance, and completion of the provider's orientation. The orientation attestation is retained for the duration of the engagement and for seven years afterwards.

Part 15 Mapping to the Strengthened Aged Care Quality Standards

This Part records how the requirements of this document give effect to the Strengthened Aged Care Quality Standards. It is used as the starting point for self-assessment and for evidence gathering before a quality audit.

Standard	Outcome	How this document gives effect to it
Standard 1	The individual	Part 1 protects the right to complain without consequence.
Standard 1	Dignity and respect	4.1 provides for apology, explanation and remedy.
Standard 2	The organisation	5.1 and 5.3 require trend analysis and independent consumer oversight.
Standard 2	Feedback and complaints	Parts 2 to 4 establish channels, timeframes, outcomes and review.
Standard 6	Food and nutrition	Complaints about food are escalated to the catering review under 5.1.
Standard 2	Continuous improvement	5.2 closes the loop by publishing what changed.

15.1 Evidence for audit

Evidence supporting each mapped outcome is maintained in the evidence library and indexed against the outcome number. Evidence must be current, dated and attributable, and must demonstrate practice rather than intention: a policy document alone is not evidence that the practice occurs.

15.2 Self-assessment

The Head of Consumer Experience completes a self-assessment against the mapped outcomes at least annually and before any planned quality audit. The self-assessment is endorsed by the Board Quality and Safety Sub-Committee and reported to the Board. The next self-assessment is due by 30 April 2027.

Part 16 Assurance, external review and regulatory engagement

Assurance over this document operates on three lines: the workers and managers who apply it day to day, the Board Quality and Safety Sub-Committee which monitors and challenges performance, and independent review commissioned by the Board. Each line is documented and none substitutes for another.

16.1 Independent review

The Board commissions an independent review of compliance with this document at least every three years, or sooner following a major non-conformity or a serious incident. The reviewer must not have been involved in writing or operating the document. The report goes to the Board unaltered, together with management's response. The next independent review must be completed by 30 September 2027.

16.2 Engagement with the Commission

The provider cooperates fully and promptly with the Aged Care Quality and Safety Commission. Requests for information are acknowledged within one business day and answered within the period specified, or an extension is sought in writing before the period expires. A worker approached directly by the Commission may speak with it freely; the provider must not require a worker to obtain permission first.

16.3 Responding to findings

Where a regulator or reviewer identifies a deficiency, the provider prepares a written response within 14 days setting out what will change, who is accountable, when it will be done and how effectiveness will be measured. Progress is reported to the Board monthly until every action is closed and verified.

16.4 Sanctions and notifiable changes

Where a sanction, a notice to agree, or a notice of non-compliance is issued, the Head of Consumer Experience must notify the Board Chair on the day it is received, and the provider must tell affected individuals and their supporters what it means for their care within five business days.

Appendix A — Definitions

The following definitions apply throughout this document.

Term	Definition
Complaint	Any expression of dissatisfaction with the provider's services, conduct or decisions, however it is expressed.
Feedback	Any comment, compliment or suggestion that is not an expression of dissatisfaction.
Complainant	The person who raises a complaint, whether or not they are the person receiving care.
Advocate	An independent person who supports someone to express their views and exercise their rights.
Open disclosure	The open discussion with a person harmed and their supporters about an incident affecting them.
Tier	The resolution pathway assigned to a complaint on triage, reflecting its complexity and risk.

Appendix B — Competency assessment criteria

A worker is assessed as competent when they demonstrate each criterion below by direct observation in the workplace. The assessor records the date, the criterion, the outcome and any remediation required.

Criterion	Evidence sought	Outcome
Receives a complaint without defensiveness	Observed role-play; complainant feels heard	Competent / Not yet
Records a complaint the same day	Register entry matches the account given	Competent / Not yet
Identifies abuse indicators in a complaint	Correctly identifies triggers in four of five scenarios	Competent / Not yet
Explains external complaint rights	Names the Commission and the advocacy service unprompted	Competent / Not yet
Arranges interpreter support	Demonstrates the booking process end to end	Competent / Not yet

Appendix C — Internal audit tool

This tool is completed at each internal audit. Each question is answered yes, no or not applicable, with a comment recording the evidence examined. A 'no' answer requires a corrective action with an owner and a due date.

#	Audit question	Evidence examined
1	Was every complaint acknowledged within one business day?	Complaints register
2	Was a risk screen for abuse and neglect completed on receipt?	Triage records
3	Where a timeframe was missed, was the complainant told before the due date?	Correspondence
4	Were written outcomes provided in plain language and translated where required?	Outcome letters
5	Were Tier 3 complaints investigated by someone independent of the area?	Investigation records
6	Were improvements published in the quarterly bulletin?	Bulletin archive
7	Were any contact restrictions approved by the CEO, time-limited and notified in writing?	CEO approvals
8	Is external complaint information displayed in every wing?	Site inspection

Appendix I — Abbreviations

The following abbreviations are used in this document and in the records it requires. Abbreviations must not be used in clinical entries where the meaning could be ambiguous.

Abbreviation	Meaning
ACQSC	Aged Care Quality and Safety Commission
ATAGI	Australian Technical Advisory Group on Immunisation
CGC	Clinical Governance Committee
CEO	Chief Executive Officer
EN	Enrolled nurse
IDDSI	International Dysphagia Diet Standardisation Initiative
MDT	Multidisciplinary team
PPE	Personal protective equipment
PRN	Pro re nata, meaning as required
RMMR	Residential Medication Management Review
RN	Registered nurse
SDM	Substitute decision-maker
SIRS	Serious Incident Response Scheme
WHS	Work health and safety

Appendix J — Worker acknowledgement

Every worker to whom this document applies must read it and acknowledge that they have done so. Acknowledgement records that the worker has read the document; it does not replace the competency assessment at Appendix B.

Acknowledgements are recorded in the learning management system. A worker who has not acknowledged the current version within 30 days of its publication is reported to their line manager, and a worker who has not acknowledged it within 60 days may not be rostered to a function this document governs until they do.

Acknowledgement statement

I confirm that I have read and understood this document, that I have had the opportunity to ask questions about it, and that I understand my obligations under it. I understand that where I am unsure how this document applies to a situation, I am expected to ask before acting rather than to assume.

Name	Role	Signature	Date

Completed acknowledgement sheets are returned to the document owner and retained for the life of the version plus seven years.

Appendix H — Implementation plan for the current version

The following milestones apply to the implementation of this version. Milestones are tracked by the document owner and reported to the committee monthly until all are closed.

Milestone	Owner	Due
Publish version 5.0 and display updated feedback information in every wing	Document owner	1 Oct 2026
Brief all workers on the three-tier pathway	Head of Consumer Experience	31 Oct 2026
Close the annual consumer experience survey	Quality Manager	30 Nov 2026
Commence weekly CEO escalation reporting	Quality Manager	1 Dec 2026
Publish the first quarterly bulletin under the new version	Communications Lead	15 Jan 2027
Complete translation of complaint materials	Communications Lead	28 Feb 2027

Appendix E — Escalation matrix

This matrix sets out who must be told, and how quickly, when a matter arises under this document. Where two rows could apply, the shorter timeframe governs.

Trigger	Escalate to	Within
Complaint disclosing abuse, neglect or unexplained injury	Facility Manager and incident process	Immediately
Complaint alleging misconduct by a senior manager	Chief Executive Officer	Within 1 business day
Complaint alleging conduct by a Board member	Board Chair	Within 1 business day
Request for internal review	Head of Consumer Experience	Within 2 business days
Proposed restriction on complainant contact	Chief Executive Officer	Before any restriction applies
Commission notifies it has received a complaint	Chief Executive Officer and Board Chair	Same day

Appendix F — Forms and templates

The following forms support this document. Forms are version-controlled and must be obtained from the intranet rather than copied from a local drive.

Form	Purpose	Retained by
HV-F-003a Feedback and complaint form	Records a complaint or compliment in the person's own words	Head of Consumer Experience
HV-F-003b Triage and risk screen	Records the tier assigned and any abuse indicators	Head of Consumer Experience
HV-F-003c Outcome letter template	Provides a plain-language written outcome	Head of Consumer Experience
HV-F-003d Internal review request	Records a request for review and its outcome	Chief Executive Officer
HV-F-003e Contact restriction approval	Records the CEO's approval, reasons and end date	Chief Executive Officer

Appendix G — Frequently asked questions

These questions arose during consultation on the current version and are reproduced with the answers given.

Question	Answer
A resident says 'I don't want to make a fuss'. Is that a complaint?	Treat it as one. An expression of dissatisfaction is a complaint whether or not the person uses the word, and whether or not they want it escalated. Record it and ask what they would like to happen.
Can I resolve something myself instead of writing it down?	Resolve it, and still record it. Tier 1 concerns are meant to be fixed on the spot, but the record is how the provider sees patterns across shifts and wings.
A family member wants to complain but the resident does not. What do I do?	Accept the complaint. A supporter may complain with or without the person's involvement. The person's own wishes about their care remain paramount in the outcome.
Someone says they will go to the Commission. Should I ask them to let us try first?	No. They may complain externally at any time, and you must not discourage or delay it. Offer to help them do so if they would like.
What if a complaint is about me?	Hand it to your manager and take no part in handling it. You will be told what is happening and supported through the process.

Appendix D — Improvement actions arising from the current review

The following actions were agreed at the most recent review of this document and are tracked in the continuous improvement register until closed.

Ref	Action	Responsible	Due
C1	Add a confidential question on fear of complaining to the annual survey	Head of Consumer Experience	30 Nov 2026
C2	Weekly escalation report to the CEO on complaints approaching their due date	Quality Manager	1 Dec 2026
C3	Publish the quarterly 'You said, we did' bulletin	Head of Consumer Experience	15 Jan 2027
C4	Translate complaint materials into six community languages	Communications Lead	28 Feb 2027
C5	Deliver refresher training on recognising abuse disclosed in complaints	Learning and Development Lead	31 Mar 2027
C6	Independent review of complaint handling quality	External reviewer	30 Sep 2027